



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : رحاب حسن محمد مجاهد
المحافظة : الفيوم الحى / المدينة : الفيوم
رقم البطاقة : 27409012300503 رقم التليفون : 01020159579
العنوان : ++++++
العمر (تاريخ الميلاد) : 01-09-1974 المهنة : حاصل علي دبلوم
تاريخ تسجيل الإستمارة : 23-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: رحاب حسن محمد مجاهد National ID: 27409012300503	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



رحاب
حسن محمد مجاهد
الصفية
اول القيوم - القيوم

٢٧٤ . ٩ . ١ ٢٣ . ٥ . ٣

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٢٠٢٠/٠٨ ، ٢٧٤٠٩٠١٢٣٠٥٠٣
حاصله على دبلوم تجارة



أنشى مسلمة
نصر محمد حسين

البطاقة مارية حتى ٢٠٢٧/٠٨/٣٠

٦





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : رحاب حسن محمد مجاهد

Sex:

MDT form

3- Medical number:
Female

83056935

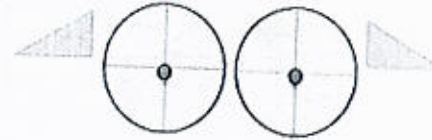
Old Med :

Old Med :

478-2021

2-ID number: 27409012300503

Patient Age : 50 Y



Family history

Family History
-ve

Phone

Phone

01020159579

BMI

BMI

REASON OF MDT

REASON OF MDT
REMDT

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP
C

Date of MDT

Date of MDT

28/05/2024

Co morbidity

Co Morbidity
-veMenopausal
statusMenopausal Status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :

BX IDC BX COMMENT:	ER 4/8 ER COMMENT:	PR 2/8 PR COMMENT:	Her 2 Neu Her 2 Neu COMMENT: -ve	SISH SISH COMMENT :	Ki67 60
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T RECURRENT PT2	N 0	M 0
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Sonomammography

PATIENT HISTORY

bilateral breast cancer 2022 : rt ct1 n0 IDC II triple -ve ki67 40%

lt ct3 m n0 IDC II ER 7/8 PR 7/8 HER2 SISH not amplified

received NACT 4 AC + 9 wks taxol

grade 2-3 PN --> bilateral BCS 13-9-2023

RT YPTis YPN0

LT YPTis YPN1

received tam . PORT

28/10/2023: RT operative bed masses

biopsy IDC GIII ER 4/8 PR 3/8 HER2 -ve KI67 60%

bone scan: free

POST rt simple mastectomy with no axillary LN excision:

metaplastic carcinoma GIII DCIS 20%

free nipple , deep margin , intramammary LN 0/3

PT2 PNO TRIPLE NEGATIVE

pt received xeloda 1 ccyles , navelbine x 4 cycles bec of unavailability of xeloda

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR adjuvant AI after serial measurement of estradiol , LH and FSH

PT refusing any further chemo

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ شريف حسين د/ مروه سليم	RADIOLOGY	RADIOLOGY د/ رانيا شافق
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY

Patient Name : الأستاذة / رحاب حسن محمد	Request No. : 13008913
Patient No. : 27409012300503	Request Date : 29/10/2023
Path No. : 3729/23	Verification Date : 06/11/2023
Sex : Female	Financial Category : أورام دار السلام - مبادره
Age : 48 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youssef

Pathology Report

Clinical data:

Right breast operative bed mass in a known case of right breast cancer post conservative surgery.

Nature of specimen:

Tru cut biopsy, collected at 29/10/2023 , 12:10 pm.

Gross examination

Multiple fragmented greyish white tissue cores up to 0.5 cm , totally submitted.

Microscopic examination:

- * Histological type: Recurrent invasive duct carcinoma .
- * Grade (differentiation +nuclear pleomorphism+mitosis): (3+3+2) Grade III .
- * DCIS: Could not be detected within the limits of biopsy recieved.
- * Angioinvasion: Could not be detected within the limits of biopsy recieved.

Diagnosis:

Right breast operative bed mass, tru cut biopsy:
Recurrent invasive duct carcinoma, Grade III .
For further IHC (ER,PR,HER2,Ki67).

Prof. Gamal Abdelatty

Dr. Menan Hossam

Verified by :
dr.esraa magdy

Printing Date : 11/11/2023 11:36:28

Page 1 of 1

Printing By : marwa mohamed youssef



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - نيل نيقوش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qudema - Cairo, Egypt

Patient Name : د. أسماء / رانيا حسن محمد
Patient No. : 27409012300503
Path No. : 3729/23
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13008913
Request Date : 29/10/2023
Verification Date : 18/11/2023
Financial Category : تأمين - مبرور
Department : Out Patient
IHC No. :
Registered by : marwa mohamed

IMMUNOHISTOCHEMISTRY REPORT

Clinical data:

Right breast operative bed mass in a known case of right breast cancer post conservative surgery.

Nature of specimen:

Tru cut biopsy, collected at 29/10/2023 , 12:10 pm.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- IPT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in 5% of tumor cells (Allred score 4/8).
- PR: Weak nuclear reaction 5% of tumor cells (Allred score 3/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 60% nuclear reaction of tumor cells.

Conclusion:

Right breast operative bed mass, tru cut biopsy:
Recurrent invasive duct carcinoma, Grade III.

IPT:

- ER: Positive (score 4/8).
- PR: Positive (score 3/8).
- Her2: Negative (score 0).
- Ki67: Positive in 60% of tumor cells.

Verified by :

Rana fathy

Printing Date : 19/11/2023 10:43:07

Page 1 of 2

Printed By : Ali mostafa



مركز الأورولوجي - مبنى (4) - الدور الأول - مبنى رقم (967) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 967 - Nile Corniche - Mar El Qadima - Cairo, Egypt



Patient Name: الأستاذة / رانيا حسن محمد
Patient No.: 27409012300503
Path No.: 3729/23
Sex: Female
Age: 48 Y
Referring Doc.:

Request No.: 13008913
Request Date: 29/10/2023
Verification Date: 18/11/2023
Financial Category: أورام دار السلام - مياطرة
Department: Out Patient

Registered by: marwa mohamed you

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Rana Fathy
Dr. Menan Hossam

Verified by:
Rana fathy

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Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / رباب حسن محمد
Patient No. : 27409012300503
Path No. : 2703/22
Sex : Female
Age : 47 Y
Referring Doc :
Request No. : 13002789
Request Date : 13/09/2022
Verification Date : 05/10/2022
Financial Category : أورام دار السلام - مياثرة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data:

Bilateral breast cancer

Nature of the specimen:

Bilateral WLE & SLNs

Gross Examination:

Four specimens were received submitted as:

- 1- Right WLE measuring 6x5x4 cm, Cut section revealed an ill-defined firm grayish white mass measuring 2x1x0.5 cm, located 1 cm from least deep margin.
- 2- Right Sentinel lymph node: fibrofatty tissue pieces measuring collectively 4x4 cm, entangling five nodules ranging in size from 0.5 cm to 1.5 cm.
- Frozen section: Free margins & Free sentinel lymph nodes (0/5)
- 3- Left WLE measuring 8x8 cm, Cut section revealed an ill-defined firm grayish white mass measuring 4x2x0.5 cm, located 1 cm from least deep margin.
- 4- Left Sentinel lymph node: fibrofatty tissue pieces measuring collectively 4x4 cm, entangling six nodules ranging in size from 0.5 cm to 1.5 cm.
- Frozen section: Free margins & Positive one sentinel lymph nodes (1/6)

Microscopic examination:

Right breast:

- Total regression of tumor tissue
- Angioinvasion: No
- Ductal carcinoma in situ: yes. 1%
- Lymph node involvement: NO (0/5)
- Resection Margins: Free
- Histologic response to therapy: Complete pathological response
- Additional pathologic findings: Hyalinosis & fibrosis (Effect of therapy)

Verified by :

Dr. Marwa mahmoud

Printing Date : 05/10/2022 11:50:15

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ملف رقمي (ID) : 0079 - رقم الملف : 0079 - رقم الملف : 0079 - رقم الملف : 0079
Rome No. 10 - First Floor Building No. 781, Via Corsica - 50139 Florence, Italy
Rome No. 10 - First Floor Building No. 781, Via Corsica - 50139 Florence, Italy



Patient Name	: الأستاذ / رهاب حسن محمد	Request No.	: 13002789
Patient No.	: 27409012300503	Request Date	: 13/09/2022
Path No.	: 2703/22	Verification Date	: 05/10/2022
Sex	: Female	Financial Category	: اورام دار السلام - مبداءه
Age	: 47 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Left breast:

- Total regression of tumor tissue
- Angioinvasion: No
- Ductal carcinoma in situ: yes. 1%
- Lymph node involvement: Yes (1/6)
- Resection Margins: Free
- Histologic response to therapy: Partial pathological response
- Additional pathologic findings: Hyalinosis & fibrosis (Effect of therapy)

Diagnosis:

Right breast cancer, Right WLE & SLNs;

Total regression of tumor tissue

Focal DCIS (1%)

Free resection margins.

Free sentinel lymph nodes (0/5)

Stage: ypTis ypN0

Left breast cancer, Left WLE & SLNs;

Total regression of tumor tissue

Focal DCIS (1%)

Free resection margins.

Positive one sentinel lymph nodes (1/6)

Stage: ypTis ypN1

Prof. Gamal Abdelaty

Verified by :

Dr. Marwa mahmoud

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Printing By : marwa mohamed youss

Patient Name : الأستاذ / رحيل حسن محمد
Patient No. : 27409012300503
Path No. : 19/2022
Sex : Female
Age : 47 Y
Referring Doc :

Request No. : 136297
Request Date : 02/01/2022
Verification Date : 03/02/2022
Financial Category : مبداءة - مبداءة
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Biomarkers (Rt side):
ER: negative nuclear reaction in tumor cells (Allred score 0/8)
PR: negative nuclear reaction in tumor cells (Allred score 0/8)
Her-2: negative membrane reaction in tumor cells (score 0)
Ki 67: 40 % nuclear reaction
Biomarkers (Lt side):
ER: Strong nuclear reaction in 50% of tumor cells (Allred score 7/8)
PR: Strong nuclear reaction in 50% of tumor cells (Allred score 7/8)
Her-2: moderate complete membrane reaction in 10% of tumor cells (score 2+)
SISH: Not amplified, (Her2/cep17 ratio ~1.5, & Her2 copy number ~3.5)
Ki 67: 30 % nuclear reaction

Prof.Dr. Gamal AbdelAty
Dr. Israa Magdy

Verified by:
Rana fathy

ing Date : 07/02/2022 09:44:08

Page 2 of 2

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معمل الباثولوجي - مستشفى قصر العيني - القاهرة - مصر
Room No. 143 - First Floor Building No. 201, New Complex - Kasr El-Eini - Cairo - Egypt

Patient Name : الأمثلة / رحاب حسن محمد
Patient No. : 27409012300503
Path No. : 3/2022
Sex : Female
Age : 47 Y
Referring Doc. :

Request No. : 136066
Request Date : 01/01/2022
Verification Date : 19/01/2022
Financial Category : Out Patient
Department :
IHC No. :
Registered by : manva mohamed y

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right & Left breast masses

Nature of the specimen: Tru-Cut biopsy.

Gross Examination:

Two specimens were received:

- 1- Right breast mass: multiple greyish white tissue cores up to 1.5 cm long, totally submitted.
- 2- Left breast mass: 4 greyish white tissue cores up to 2 cm long, totally submitted.

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2 ER clone (SP1), PR clone (1E2), HER-2 CLONE (4B5), & Ki67 (30-9)

Microscopic examination:

Right & Left breast masses:

- Histological type: Invasive duct carcinoma
- Grade: II (differentiation 3 + pleomorphism 2 + mitosis 1)
- Angiolysis: N/A
- Ductal carcinoma in situ: focal
- Tumor extent: N/A
- Lymph node involvement: N/A
- Resection Margins: N/A
- Additional pathologic findings:

- Focal necrosis (in left).
- No microcalcification.

Right breast IPT:

- Examination of ER stained slide show no nuclear staining in tumor cells.

- Examination of PR stained slide show no nuclear staining in tumor cells.

- Examination of HER2 stained slide show no membranous staining in tumor cells.

Verified by:

dr.esraa magdy

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Patient Name الأستاذة / رحاب حسن محمد
Patient No. 27409012300503
Path No. 3/2022
Sex Female
Age 47 Y
Referring Doc.

Request No. 136066
Request Date 01/01/2022
Verification Date 19/01/2022
Financial Category أورام دار السلام - مياثره
Department Out Patient

Registered by marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

- Examination of Ki67 stained slide show nuclear staining in 40% of tumor cells.
- Left breast IPT:
- Examination of ER stained slide show intense nuclear staining in 70% of the tumor cells.
- Examination of PR stained slide show moderate nuclear staining in 70% of the tumor cells.
- Examination of HER2 stained slide show intense staining in 10% of tumor cells with incomplete membranous staining.
- Examination of Ki67 stained slide show nuclear staining in 30% of tumor cells.

Diagnosis:

Right & Left breast masses, Tru-Cut biopsy:
- Invasive duct carcinoma GII.

- Right biopsy biomarkers:

*ER: Negative (0/8)

*PR: Negative (0/8)

*Her2: Negative (Score 0)

*Ki67:40%

- Left biopsy biomarkers:

*ER: Positive (7/8)

*PR: Positive (7/8)

*Her2: Equivocal (Score 2+) for SISH

*Ki67:30%

Prof.Dr. Gamal AbdelAty
Dr. Israa Magdy
Dr. Walaa Ghonaim

Verified by :

dr.esraa magdy

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معمل الباثولوجي - مبادرة صحة المرأة - معمل الباثولوجي - معمل الباثولوجي - معمل الباثولوجي
Room No. (42) - First Floor - Building No. 231 - New Corniche - Near El-Qadima - Cairo
Kafan

Patient Name: Rehab Hassan Mohamed Mogahed	Date: 02/05/2024
Exam: C.T CHEST	Age: 49 Year(s)
Medical ID: 478/2022	Pat. ID: 16624

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -
Compared to the external study dated 25/12/2023; the new one revealed:

1. **Chest:**
 - Interval right mastectomy with scarring at operative bed and right axillary fold as well as minimal skin thickening yet with no gross sizable masses.
 - Stable left upper lung lobe small nodule reaching about 4 mm.
 - Middle and lingular lung lobes subpleural reticular infiltration and atelectasis, mostly Post irradiation sequel.
 - No pathologically enlarged hilar or mediastinal lymph nodes.
 - No evidence of pleural effusion, pleural thickening or masses seen.
 - Average cardiac size. No pericardial effusion detected.
2. **Abdomen and pelvis:**
 - Enlarged liver shows diffuse low fatty attenuation. No definite focal lesions or contour irregularities.
 - No intra-hepatic or extra-hepatic biliary radicles dilatation.
 - Normal size & parenchymal texture of the spleen with no focal lesions seen.
 - No pathological abdominal or pelvic lymph nodes.
 - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
 - Normal appearance of the pancreas and both supra-renals.
 - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
 - Normal appearance of the other pelvic organs.
 - No abdominal or pelvic masses or collections.

Patient Name: Rehab Hassan Mohamed Mogahed	Date : 02/05/2024
Exam : C T CHEST	Age : 49 Year(s)
Medical ID: 478/2022	Pat. ID: 18624

3. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- Follow up status with interval right mastectomy.

showing:

- Stable left pulmonary nodule.
- No definite hepatic metastatic deposits on non contrast basis.

Dr.Shaimaa Megahed, MSc.

Dr. Mohammed Mofsen, M.D.

Patient's Name: REHAB HASAN MOHAMED

Date: 2026-04-14

Referred by: Prof.Dr.

Hospital: Goustave Roussey International Egypt

Left Digital soft tissue mammography with high resolution sonography

revealed:

Clinical summary: Status post right MRM , Left CBS & bilateral ALAND routine follow up.

Findings:

Right mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- Thickened skin and coarsen trabeculae.
- UOQ architectural distortion with no speculations, masses, asymmetry , or suspicious microcalcifications.
- **Ultrasound** demonstrated UOQ area of parenchymal disruption underneath the operative scar with no solid , cystic lesions or fluid collections.
- Thickened skin reaching 5 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Clear left axilla. No pathological infra or supra-clavicular lymph node

Conclusion:-

Clear right mastectomy bed and axilla.

Left breast (BIRADS 2): Post therapeutic sequelae.

Recommendation:-

For annual follow up.

Best Regards,
DR Amal Hegazi, M.Sc.

Dr Amal Hegazy

Dr Amal Hegazy